



P22 · 32 PAGES, AGES 4 TO 12 MONTHS

## *An Introducing Solids Planner*

*Thirty-two pages of plain, no-judgment guidance for the first months of eating. Readiness signs, the gear list, a daily-tracker template, the twelve-food list that covers the bases, an allergen-introduction log, scripts for the grandparent who keeps offering things, and a "what if they hate it" page that does not panic.*

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*Soothemade* **Notes**

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY

## A note before you start

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The first time I tried to feed my baby a real food, I had read three books, watched four YouTube videos, and was wearing a clean shirt. By the end of the feed, the sweet potato was in his hair, on my arm, on the dog, and on the back of the chair. He had eaten approximately one teaspoon. I considered this a profound failure.

It wasn't a failure. That's actually most of what a first solids feed looks like. The food on the body, on the surroundings, on the floor. That IS the eating, for a while.

This planner is the version of the conversation I wish I'd had on the kitchen counter. Not a book about baby-led weaning versus purees. Not a chart of what nutrients you must hit by month seven. Just a planner: when, what, how, and what to do when something goes sideways.

Soothemade

*This is a planning tool, not medical or nutritional advice. If your baby has feeding concerns, weight concerns, allergies in the family, or any complexity, please talk to a pediatric provider. The numbers are at the back.*

# The readiness page

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You're looking for four to six signs in the same month. Most babies hit them between 4 and 7 months. Some hit them at 5. Some at 8. The window is wide.

## PHYSICAL

- ☐ Sits with support, head steady
- ☐ Has lost the tongue-thrust reflex (food no longer pushed straight back out)
- ☐ Can bring objects to the mouth
- ☐ Has roughly doubled their birth weight

## BEHAVIORAL

- ☐ Watches you eat with intense interest
- ☐ Opens mouth when food approaches
- ☐ Reaches for your food
- ☐ Makes chewing motions even without food in the mouth

## *A note on the four-vs-six month conversation*

Older guidance said "wait until six months." More recent guidance (AAP, WHO) says any time between 4 and 6 months is fine if the baby is showing readiness, with a slight emphasis on closer to 6.

If your pediatric provider has given you a different specific time, follow their guidance over this planner. If your baby was premature, the adjusted age matters more than the calendar age.

# The gear list

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The six things you need:

- ❑ One small spoon (silicone-tipped is good)
- ❑ A bib that catches (silicone with a pocket is best, but a cloth one works)
- ❑ One small bowl (one that won't break when it falls)
- ❑ A high chair (or a booster on a real chair) with footrest support. The feet matter for swallowing.
- ❑ A mat or splat cloth under the high chair
- ❑ Wipes within arm's reach

Five things you do NOT need:

- × A baby-food processor (your existing blender is fine)
- × A subscription baby-food service (homemade is fine, store-bought is fine)
- × A weighted spoon (just a small spoon)
- × A baby-food cookbook (the internet has free recipes, the food is mostly just-soft-mashed-something)
- × A complex schedule (the first three months are mostly exploration, not nutrition)

## The daily eating log

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You'll see this template in the planner for the first 60 days. Use it for as long as it's useful.

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DATE \_\_\_\_\_ / BABY'S AGE: \_\_\_\_ months \_\_\_\_ days

| TIME | WHAT WAS OFFERED | REACTION | AMOUNT |
|------|------------------|----------|--------|
|------|------------------|----------|--------|

|       |       |       |       |
|-------|-------|-------|-------|
| _____ | _____ | _____ | _____ |
| _____ | _____ | _____ | _____ |
| _____ | _____ | _____ | _____ |

REACTION KEY: ☐ ate eagerly ☐ tried, then stopped  
☐ refused ☐ played with it  
☐ allergic-watch (rash, swelling, vomiting)

AMOUNT KEY: ☐ trace ☐ teaspoon ☐ tablespoon  
☐ small bowl ☐ ate most of it

ANYTHING NEW TODAY (texture, food, behavior):

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THE MILK FEED RHYTHM TODAY:

Number of breast/bottle feeds: \_\_\_\_

Were milk feeds shorter than usual? (circle): yes / no

Was the baby hungry between solid feeds? (circle): yes / no

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The point of the log is not to score the baby. It's to spot the pattern. After two weeks, you'll see what they actually like, what they refuse, and what shifts the milk-feed rhythm.

# The twelve-food starter list

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**A** loose three-month sketch. Not a rule. Skip foods that don't apply (allergies in your family, cultural diet, what you have on hand).

## *First foods (weeks 1-4 of solids)*

Simple, single-ingredient, easy to mash or hold.

1. Avocado (soft, fatty, almost no prep)
2. Banana (very soft, naturally sweet)
3. Sweet potato (steamed or roasted, mashed)
4. Cooked carrots (steamed soft, mashed or sliced into finger-shape strips)
5. Pear (very ripe, peeled, mashed)
6. Cooked apple (steamed or microwaved soft, mashed)

## *Second foods (weeks 5-8)*

Slightly more variety, introducing the common allergens one at a time.

7. Peanut butter (thinned with water or breast milk, the spoonful-on-toast version, NEVER a whole spoonful of stiff peanut butter)
8. Yogurt (whole-milk, plain, full-fat)
9. Egg yolk (cooked solid, mashed)
10. Iron-rich food (lentils, soft-cooked beans, ground meat in a sauce, fortified baby cereal)

## *Third foods (weeks 9-12)*

Building texture and variety.

11. Soft pasta (small shapes, plain, with a little oil)
12. Soft chicken (shredded fine, in a broth)

After this, the world opens up. By month 9, most babies are eating versions of what the family is eating, mashed or chopped soft.

## *The allergen-introduction log*

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The “big 9” common allergens are: peanuts, tree nuts, eggs, milk, wheat, soy, sesame, fish, shellfish.

Current guidance from the AAP is to introduce common allergens early (around 6 months for most), one at a time, with a few days between each, so any reaction can be traced.



There is a specific exception: if your baby has severe eczema, a known food allergy already, or a strong family history of food allergies, talk to your pediatric provider before introducing allergens, especially peanut. They may want to test or guide the introduction specifically.

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#### ALLERGEN INTRODUCTION LOG

| ALLERGEN | DATE | REACTION OBSERVED<br>(over 3 days after) |
|----------|------|------------------------------------------|
|----------|------|------------------------------------------|

|                    |       |                                                              |
|--------------------|-------|--------------------------------------------------------------|
| Peanut             | _____ | <input type="checkbox"/> none <input type="checkbox"/> _____ |
| Tree nuts          | _____ | <input type="checkbox"/> none <input type="checkbox"/> _____ |
| Egg                | _____ | <input type="checkbox"/> none <input type="checkbox"/> _____ |
| Milk (cow's)       | _____ | <input type="checkbox"/> none <input type="checkbox"/> _____ |
| Wheat              | _____ | <input type="checkbox"/> none <input type="checkbox"/> _____ |
| Soy                | _____ | <input type="checkbox"/> none <input type="checkbox"/> _____ |
| Sesame             | _____ | <input type="checkbox"/> none <input type="checkbox"/> _____ |
| Fish (e.g. salmon) | _____ | <input type="checkbox"/> none <input type="checkbox"/> _____ |
| Shellfish (later)  | _____ | <input type="checkbox"/> none <input type="checkbox"/> _____ |

WATCH FOR (within minutes to 2 hours of eating):

- ☐ Hives or red blotches
- ☐ Swelling of lips, eyes, or face
- ☐ Vomiting that is sudden and forceful
- ☐ Difficulty breathing or wheezing
- ☐ Sudden fussiness, paleness, or limpness

Any of these = call your provider or 911 immediately.

WATCH FOR (over the next 1-3 days):

- ☐ Eczema or rash that wasn't there before
- ☐ Diarrhea that wasn't there before
- ☐ Mucus or blood in stool
- ☐ Sudden refusal to eat

These are slower-onset reactions. Worth a call.

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# The “they hate it” page

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The first time most babies meet a new food, they make a face. The face is not a verdict. It is “what is this thing on my tongue.”

## *What’s typical*

- Sticking out the tongue
- Making a sour-face even with sweet foods
- Spitting it back out
- Crying briefly
- Refusing the second bite

All of these can mean “I’m still learning what to do with this.” Not “I hate it forever.”

## *What helps*

- **Offer the same food 8-15 times** before deciding the baby doesn’t like it. (Eight to fifteen, not one.)
- **Try a different texture.** A baby who refuses mashed avocado might happily eat slices of it.
- **Try at a different time.** A baby who refuses food at 5pm might eat it at 10am, when they’re more rested.
- **Eat the food in front of them, with your own spoon, with theatrical enjoyment.** This is not silly. It works.

## *What does NOT help*

- Forcing the spoon into the mouth (creates an aversive memory)
- Hiding food in another food (works briefly but stops working)

- Withholding milk to make them hungrier for solids (the milk is still the main calorie source until at least 8-10 months)
- Skipping meals as punishment for refusal (babies don't connect those dots)

### ***When to actually worry***

- The baby is not gaining weight at appointments
- The baby is choking (not gagging, choking, see the safety page)
- The baby has a real allergic reaction (see the allergen log)
- The baby refuses ALL solids for 3+ weeks, AND is not gaining

In any of those cases, call your pediatric provider.

# The “purees vs finger foods” page

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A page you can read once and stop wondering about.

## *Both are fine*

There is no “better” between purees and baby-led weaning (BLW, the all-finger-foods approach). Both end in a kid who eats. The research does not support one over the other.

Many families do both. A spoonful of mashed sweet potato AND a piece of steamed carrot, on the same tray. Babies tend to like the choice.

## *What purees are good for*

- Iron-rich foods that don’t naturally come in finger-food form (pureed lentils, pureed meat in sauce)
- The first weeks of figuring out how a spoon works
- Travel or rushed mealtimes
- Babies who are very small or have feeding concerns flagged by a provider

## *What finger foods are good for*

- Hand-eye coordination
- The baby setting their own pace
- Exposure to texture
- Mealtimes where the family is eating together and the baby eats versions of the same food

## *Safety, for both*

- **Sit upright.** Always. Never feed a reclined baby.

- **Feet supported.** A footrest, a box, anything that gives the feet a surface to brace on. Feet matter for swallowing.
- **Adult-supervised.** Eyes on the baby for every bite, every meal.
- **Honest size cuts.** Anything round (grapes, cherry tomatoes) is cut in quarters, the long way. Hot dogs (which a baby this age shouldn't have anyway) are never cut in rounds, ever.
- **No hard foods.** Raw apple, raw carrot, whole nuts, popcorn, hard candies. None until age four for the highest-risk foods.

### *Choking versus gagging*

- **Gagging is normal.** It is the body's protective reflex. It looks alarming. It is not an emergency. The baby coughs, often re-positions the food, and continues. Resist the urge to scoop the food out (it usually moves it deeper, not out).
- **Choking is silent.** The baby cannot make a sound. The face turns blue or red. This is when you act fast. Please take an infant CPR class. Look up "first-aid for infant choking" before you start solids. The technique for babies is specific and different from adult Heimlich.

## *The pumping companion's note*

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**I**f you are also breastfeeding or pumping, solids do not replace milk yet.

- Months 4-6: milk is still 100% of nutrition. Solids are exposure and practice.
- Months 6-9: milk is still most of nutrition. Solids are growing.
- Months 9-12: solids are roughly half. Milk is still around.
- Month 12+: the ratio inverts. Milk becomes the smaller portion.

The general rule: offer milk BEFORE solids in the first months. The milk is the calories. The solid is the lesson.

(This shifts around 9-10 months for many families. Your pediatric provider will guide.)

## *The mealtime scripts*

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**F**or the moments when other adults are involved.

### ***For the grandparent who wants to offer pre-chewed food***

"We don't do pre-chewed food in our family. The adult mouth has bacteria a baby isn't ready for. I know it was common, and I'm sure you meant well. We'll skip it from now on."

### ***For the in-law concerned about salt***

"We do hold the salt for the baby's portion, yes. Their kidneys are still developing. We salt our food at the table."

### ***For the relative offering honey***

"Honey is a no until age one. Botulism risk. They can have it on their first birthday."

(Honey IS dangerous for babies under 12 months. This is a real safety thing, not a preference.)

### ***For yourself, when the baby has refused the same food a third week in a row***

"This is the seventh time I've offered this. It's normal. I will keep offering. Today doesn't count."

### ***For your partner, when the kitchen is chaos***

"Solids time is messy. The mess is the lesson. I'll wipe later. Just stay calm."

## *A monthly check-in*

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A page for the end of each month, for the first six months of solids.



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END OF MONTH \_\_\_\_ OF SOLIDS

THE BABY (this month)

New foods they tried:

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Foods they ate at least 3 times:

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Foods they refused 3+ times (don't drop yet):

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Allergen reactions (any):

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Approximate "real food" intake at the average meal:

- ☐ traces   ☐ a few bites   ☐ small bowl
- ☐ small adult portion

Milk feeds (per day, average): \_\_\_\_

Weight at last check: \_\_\_\_ (date: \_\_\_\_)

ME (this month)

The hardest part of feeding this month:

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The unexpected easy thing:

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What I want to try next month:

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Did I take any photos of the mess? (yes, take more)

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## The “I’m overwhelmed” page

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Some weeks of solids are surprisingly hard. The kitchen never feels clean. You are tired. The baby is teething. They threw the bowl on the floor for the fourth meal in a row.

A short page for those weeks.

### *The simplest possible meal plan, for a tired week*

#### MORNING

- ☐ Half a piece of toast, soft, with smooth nut butter
- ☐ A mashed banana

#### MIDDAY

- ☐ Plain yogurt with a soft fruit stirred in
- ☐ A pre-cooked steamed carrot stick

#### EVENING

- ☐ Whatever you are eating, in small soft pieces

You can do this rotation for a week. The baby will be fed. You can think harder about variety next week.

### *Things you can skip this week*

- Variety (one or two foods per meal is fine)
- Photos (the moment will still happen, photographed or not)
- Pinterest meals (those are content, not feeding)
- New foods (postpone the next allergen for a few days)
- The high chair tray being clean before the next meal

## *Things you cannot skip this week*

- Sitting upright while eating
- Adult supervision
- Honest size cuts (no choking hazards)

The list of “cannot skip” is short. Hold the line on that one. Everything else can flex.

# Numbers to keep

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## PEDIATRICIAN

Name: \_\_\_\_\_ Number: \_\_\_\_\_

After-hours line: \_\_\_\_\_

## PEDIATRIC ALLERGIST (if relevant)

Name: \_\_\_\_\_ Number: \_\_\_\_\_

## PEDIATRIC FEEDING SPECIALIST (if needed)

Name: \_\_\_\_\_ Number: \_\_\_\_\_

## LACTATION CONSULTANT (still useful in solids)

Name: \_\_\_\_\_ Number: \_\_\_\_\_

## POISON CONTROL (US)

1-800-222-1222 (24/7)

## LOCAL ER

Hospital: \_\_\_\_\_ Number: \_\_\_\_\_



From *Soothemade Notes*, a small apothecary of printables, planners, and cards for the unphotographed parts of new parenthood. Made slowly, in plain language.



# Soothemade *Notes*

*Made for the version of you no one tells you about, the  
one at four in the morning, the one in the parking lot, the  
one who doesn't recognize her own week.*

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